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Press Conference - Australian Parliament House

Prime Minister

PRIME MINISTER: Good morning, afternoon, I should say. I'm joined by the Minister for Health, I'm joined by the Secretary of the Health Department and the Chief Medical Officer. This afternoon we will be providing an update and making some announcements on the vaccination rollout strategy. Before I do that, though, I want to make a couple of comments regarding the rather disturbing scenes in the United States. As I've already expressed today earlier, and as so many around the world have, the riots and protests that we've seen in Washington D.C. have been terribly distressing. They are very concerning. As a result, we are making some changes to our travel advice, which is as follows: Due to violent protests in Washington D.C., curfews are in place in the District of Columbia and the Commonwealth of Virginia from 6.00pm on Wednesday the 6th of January to 6.00am on the Thursday the 7th of January and that you should avoid areas where protests are occurring due to ongoing potential for violence. Follow the instructions of local authorities, including curfews, and stay away, stay at home orders, monitor the media for information and updates. We also note that COVID-19 remains a serious health risk in the United States. Various restrictions and public health measures are in place and they vary by location. So please follow the instructions of local authorities, including those related to the quarantine, self-isolation and social distancing and the wearing of masks. Please monitor the Embassy website for further COVID-19 related information. I'm pleased that the Senate has been able to recommence their proceedings and we hope for a peaceful and stable transition of government to the new administration, elected by the American people. And this is a difficult time for the United States, clearly. They are a great friend of Australia and they're one of the world's greatest democracies. And so we just, our thoughts are with them and we hope for that peaceful transition to take place.

But to the purpose of today's calling of this press conference, we are still winning, but we have not yet won the fight against the pandemic. And that will continue to be a fight that we will wage over the course of 2021. In the most recent outbreaks over the summer, the systems have been put to the test in New South Wales, in Victoria, and today in the case that has been announced in Queensland will similarly present a test. But we have seen that test passed, not just by the systems that have been built up by those jurisdictions as we have battled that virus throughout the course of 2020, learned the lessons, improved the systems, upgraded, working together. But it is also a test that is increasingly and continues to be passed by the Australian people. Whether those up in the northern beaches of Sydney or the suburbs of Melbourne or indeed in other parts of the country, we say thank you for your patience, for your perseverance and your cooperation. And we will continue to work through those issues as we battle that virus, as we have been so successfully over the course of now 12 months. Almost 12 months ago, was when the then Chief Medical

Officer, Professor Murphy, came into my office and briefed me on the situation regarding the coronavirus at that time.

So we will continue to put our trust in those systems that have been built up and we commend everyone who is doing that job out there today, getting tested, conducting the tests, getting the reports back to people, seeking to minimise the disruption to their lives as much as possible, whether at the cricket in Sydney or they're at home or in isolation or whether they happen to find themselves. Including, we are mindful of those in Western Australia at present who are fighting fires north of Perth. I've been in contact with the Premier today and they are on top of that situation. There's been no request for federal assistance on that matter, but the Premier is aware of the availability of support if they should need it.

The vaccination policy that was adopted by our Federal Cabinet in November and subsequently also endorsed by the National Cabinet, has been put into action in these months since then, as we prepared for the rollout of the vaccine. This is ultimately a partnership between the Federal Government. It's a federal vaccination policy. But with all vaccines, they are done in partnership with the states and we've been following those processes. This is not the usual vaccination arrangement that we'd have, given its significance. And on this occasion, the federal element of this vaccination policy has been driven very much by the Health Secretary who joins us today.

The vaccination in 2021 is a key component, obviously, of how we're dealing with the pandemic here in Australia. Throughout the course of dealing with this pandemic, we have been dealing with this in a very Australian way. Of course, we are mindful of the experiences and the lessons that we can learn from other jurisdictions. Over the course of the past year, I've probably had more discussions with leaders of other countries than Australia has seen for a very, very long time. And I've found all of those engagements, as has occurred also by the Health Minister, indeed our other health officials, other chief medical officers, whether in our own region, in the Indo-Pacific or more broadly, further afield in the United States, Canada and of course, throughout Europe and the UK. But Australia has been making its own way through this, and we've been tailoring our response to our conditions, our challenges and our needs, and we have had great success by comparison. It is a terrible tragedy that there are countries in the world today who are seeing daily death rates from COVID-19 that are higher than the total number of deaths that Australia has sadly experienced in the course of the past year. And I must admit, when I reflected on that figure in the last couple of days, it was incredibly sobering. More deaths in one day from COVID in many jurisdictions, than Australia has experienced in the last year.

So we find ourselves in a different situation, a situation not unlike countries, whether it be like New Zealand, South Korea or Japan. However, I note in recent times they've also seen an increase in cases. So we will continue to make our Australian way through this COVID-19 pandemic and seek to continue to achieve the same success that we have had to date and manage the disruption as well as we possibly can, both to our economy and the daily lives of Australians. Our officials have been moving swiftly and safely to introduce the vaccine here in Australia as soon as is safely possible. Doing that is critical to public confidence in the vaccine. We have set out cautious timetables and outlined them to you over months now, and the Health Minister has engaged most significantly in communicating that to the Australian people. But behind the scenes, the officials you see here have led a process with their counterparts around the country, but particularly within the Federal Health Department, to ensure we are seeking to better those timetables. We don't want to make promises that we can't keep. That is incredibly important. We will tell you timetables when we can have confidence in those timetables and we will continue to update those timetables as more information is known and as improvements continue to be made. We know what we know and will base our information and our timetables on that rather than speculating.

There have been no delays in the introduction of the vaccine in Australia. There has been the necessary swiftness that has been asked of the TGA and, of course, the health officials that are driving this process. It is moving considerably faster than normal vaccination approval processes would occur in Australia, but without skipping a step, without cutting a corner, ensuring that everything that needs to be ticked is ticked along the way. But the priority that I and the Health Minister have placed upon this vaccination is to ensure that that is the goal. That is the task. That is the major thing that needs to be worked on by the TGA and

by our health officials to move swiftly to have this in place. So there has been no delay and no deferral. This is going as quickly and as safely as possible.

Our health officials and experts, led by Professors Murphy and Kelly, have taken us through this process again this morning and the work that's been done over this summer break. And I want to thank all of those officials who have been working incredibly hard, particularly over these last several weeks, to get things in place to advise what we can today. After considerable effort, including with our vaccine suppliers, we are now in a position where we believe we'll be able to commence vaccinations of high priority groups in mid to late February. So we're talking about next month. This will, of course, remain conditional on a number of important factors, most importantly, that final TGA approval and the delivery of the vaccine from our suppliers. To step this out, the Health Minister will go through a presentation and that will be followed by Professor Murphy and Professor Kelly. The Therapeutic Goods Administration we are hoping to secure all the data, the final data, which they've been accumulating on an incremental basis now for some weeks to have that mid this month. That is from Pfizer, I should say. We then envisage to be hopeful of an approval subject to all the data and issues that may arise from that being resolved by the end of January. AstraZeneca, we expect that process to be completed in February, but we cannot give you a closer timetable around that. Pfizer's global protocols require approximately two weeks for delivery post approval of the vaccine. So the vaccine does not arrive pre approval. The vaccine is only released and distributed to countries once the approval is provided.

Now, you remember, this is a vaccine that has to be stored at sub-70 degrees Celsius. And that obviously has logistical issues. And that is what is addressed by Pfizer. And so that is around about a two week timeframe. Outside of that, there's about another up to a week which is required for batch testing of those received vaccine doses.

There are many other issues which Professor Murphy will also speak to, which go to the distribution of the vaccine. There is the registration process, one of the issues that is already presenting itself is there can be confusion around the vaccination process and who has received what vaccines. It's very important that is crystal clear and a process has been established already to ensure that that certification is in place. And so everyone will know who has received what vaccines, because once you get the first dose within a couple of weeks to a month, you also need to receive the second dose. And so it is not just one shot here, it's a two shot process. And that has to be managed particularly for the priority populations.

There will be five phases of priority populations as we work through over the course of this year to administer the vaccine, both the Pfizer vaccine and the AZ vaccine. Today, we are going to talk about the process of those first two phases that will provide the most necessary ring of containment and protection for the Australian population. Those populations in that first phase are quarantine and border workers, front line health officials, as well as those working in aged care and disability care and those in aged care, and disability care residents. We anticipate optimistically that we would hope to start the vaccination with around 80,000 vaccinations a week. That's what we are targeting. And then seeing that build up over the next 4 to 6 weeks and we hope by the end of February- end of March, I should say, to have reached some 4 million population. That is a target. That is what we are working to. But as the information and other things become clearer, then I think we can provide further updates on how we are tracking against those goals.

We will continue to work with the states. That has been something that has been happening now for weeks and weeks, and that will continue. Tomorrow the National Cabinet will be meeting by video link and will be able to update that further and we look forward to that process with them. This will include the necessity for state governments and territory governments to harmonise and ensure a national consistency in public health orders, which is the process by which any requirement to have a vaccine is made legal across Australia. So that is a matter that I have already spoken to the Secretary of the Health Department, Secretary Murphy, Professor Murphy, to ensure that he's working with his counterparts in the other states and territories, that we can get a consistent standard of public health orders at the state and territory level, which relate to the administration of the vaccine and where if in any cases there is a requirement to have that vaccine. But that is a discussion still to be had with the states and territories and working through the process with Professor Murphy.

Finally, vaccination is not a silver bullet. Once the vaccination starts COVIDSafe practises do not end. They continue. COVIDSafe practises will be a 2021 lived experience, they will continue. The vaccine both here in Australia and around the world, will continue to be rolled out, but it will still be a fight over the course of 2021. But this will add a very, very significant further defence and offence I should also say, in combating the virus here in Australia and around the world, there is still a lot still to be learnt about these vaccines and how they impact, that is in terms of transmissibility and issues of that nature. And that's why COVIDSafe behaviours and other arrangements will still be necessary over the course of this year.

Now, just on National Cabinet, it was earlier this week I asked the Chief Medical Officer to consider a set of proposals that they could bring forward to National Cabinet. As I've said to you earlier, where there is a clear proposal for National Cabinet to consider, then, of course, very happy to convene National Cabinet. And that proposal is further being worked on, literally as we speak AHPPC is meeting now and Professor Kelly will be rejoining that meeting later today. It's being looked after by the Deputy Chief Medical Officer, Professor Kidd, right now. And they will work through those proposals, which will deal with the end to end process on international arrivals, right through that quarantine process from point of embarkation at the source country from where people are coming right through to the acquitting of the final quarantine period. And so we'll talk more about that tomorrow. I don't have anything further to add to that today. I think it's fair to leave that consideration to the medical expert panel. I want their medical advice and they are working through providing that. And we'll consider that in the morning with all the other Premiers and Chief Ministers. And I'll be back here tomorrow around about the same time to update you on those issues.

With that, I'll hand you to the Health Minister. Greg's doing a fabulous job.

THE HON. GREG HUNT MP, MINISTER FOR HEALTH: Thanks very much, thanks. Thanks PM, Professor Murphy, Professor Kelly.

If we could have the first slide, please. Today is a very important day in the course of the pandemic in Australia. We see over here the first wave, the second wave, which was overwhelmingly, of course, within Victoria, and then the outcome from the twin outbreaks in New South Wales and Victoria. Today's Australian numbers are; zero cases in the community in 7 out of 8 states and territories, 1 in Queensland, and we have full confidence in their ability to track and trace, but 0 cases in each of New South Wales and Victoria in the community, 1 expected tomorrow in New South Wales, but zero today. And it's important to remember there were, there was a prediction, for example, of 3,000 cases in New South Wales on the 8th of January made only two weeks ago, and today: zero. What that means is that the outbreak and containment structures that had been developed prior to the pandemic but enhanced during the course of the pandemic are succeeding, are protecting Australians, are saving Australian lives. Equally, it's important to understand there are now over 11 and 3 quarter million tests that have been done for COVID in Australia and only yesterday over 100,000 tests conducted in Australia with later figures to come for the day.

If I could have the second slide, please. Then we look at Australia in comparison with the world, and we see here that there are a group of countries that are highly advanced, developed countries that are living in the most difficult of circumstances, 87 million cases, over 1.88 million lives lost officially and no doubt higher for those that were never diagnosed. In particular today, it's likely that the world will see over three quarters of a million cases, over 13 and a half thousand lives lost. In the UK alone, in a country with which we're so close, over 62,000 cases and a 1,000 lives lost. And in the United States, over 240,000 cases and 3,700 lives lost. Unimaginable numbers and agony that those communities are facing. We have been fortunate through preparation and planning and the good work of Australians and our health officials and governments across Australia, all of the work, of all of the Australians that we are in a group here with Japan and Korea slightly above now, Australia, New Zealand. We know that the jurisdiction of Taiwan has also done very well, and that has framed how we approach vaccination in Australia. And it's a very important part of it. Those countries which are here are working to an expedited but full safety assessment. Others in the grip of extremes, in the dark of a northern winter are having to make the most difficult decisions around emergency vaccination, which we recognise and support and acknowledge. And given the circumstances, that's completely justifiable.

I would note that as part of our response, just to give a brief update with regard to aged care, asymptomatic testing, 67 facilities recently completed in New South Wales, no positive cases. Victoria, 256

facilities at the Commonwealth's request tested, no positive cases, all negative so far. So that then brings me to the third slide and the strategy. In particular, and I'll let Professor Murphy and Kelly speak to the bulk of this. There are 4 principles to what we're doing, as the Prime Minister said safety first, above all else. Secondly, safety is about confidence and vaccine confidence leads to high take up. There have been parts of the world that have had challenges with vaccine take up, even though they are in the grip of the most extreme phase of the pandemic. So confidence about safety is fundamental to Australians. Thirdly, swift but safe, and fourthly under promise, but over deliver. We've continuously exceeded our targets and that's what we will strive to do.

So in terms of the priority groups and this is something that's been determined by the medical experts, you will see the phases one and two. There's no surprises as the Prime Minister said, in phase 1A, quarantine and border workers, frontline health care groups, aged care and disability staff and residents. And that's been foreshadowed. And Brendan will take you through the other elements. Also though within our strategy, a very important thing is that we- they have separated on medical advice, the cold chain and the non-cold chain distribution processes. So there will be a series of hubs for the cold chain. So there is no risk of confusion about vaccines or risk to the process. And these 30 to 50 hubs will provide early support across those three groups. And then for the AstraZeneca vaccine and in turn, in time if required, for the Novavax vaccine, over a thousand locations around Australia. So over a thousand points of presence working in conjunction with the states for vaccine distribution.

Brendan?

PRIME MINISTER: Thank you, Professor, to outline the plan Brendan.

PROFESSOR BRENDAN MURPHY, SECRETARY OF THE DEPARTMENT OF HEALTH: Thanks PM, and thanks Minister.

So as the PM and the Minister have said all the advice we have in Australia is because our strong public health position is what it is. We should go with full, proper registration of these initial two vaccines. We have two of the most promising vaccines that are coming our way very shortly, but we want to have full registration. The registration process through the TGA has been extraordinarily expedited. What used to take several months is being done in days and weeks, but without any compromise, without any compromise on safety. So these processes are going really quickly. But the TGA needs a lot more information than the data that those countries that needed emergency authorisation required for that. So we need more data. We need to get that approved by our experts. And as soon as we have that data, the panels will meet that- the data will be assessed and we will get registration. As the Prime Minister has said, we're likely to have registration for the Pfizer vaccine this month, later this month. And the AstraZeneca vaccine, by the time we've got all the right data, answered all those questions, in February. That means that we can, as the Prime Minister said, assuming all those things go well, in mid to late February, we can start our phase one rollout with likely to be the Pfizer vaccine for this priority population. Which will be quarantine and border workers, frontline health care workers, residential aged care and disability staff and residents, and that will be a relatively focussed rollout, which I will come to a little bit later on and discuss how that will be done. Then when we get access to significantly more vaccine, as we release the batches of the hopefully registered and fully approved AstraZeneca vaccine, and remember, we are manufacturing this onshore, so we have guaranteed supply line of this vaccine and that will lead us to a rapid ramp up within weeks of that initial start. And we will expand the roll out to a significantly broader range of, again, at risk population. And that will include the people who are of advanced age where we know the risk is higher. Indigenous Australians over 55, who are clearly at higher risk of disease, and a number of other people with clinical conditions that make them at higher risk of COVID and a number of critical and high risk workers who are much more prone to be exposed to COVID, that ramp up will continue. And as the Prime Minister has said, by the end of March, we hope to have at least 4 million people- 4 million doses, and then we will ramp up further and expand the population more broadly.

I won't go into the details of those subsequent phases at the moment, but we'll just take you to the next slide now. So this just outlines that very first priority population, that Phase 1A in some more detail. We know we've seen the tragic consequences of outbreaks in residential aged care. So aged care facilities and the workers who can spread the virus generally will be a high priority. And that will be done by the

Commonwealth contracted officials working in partnership with the states and territories. And I do want to emphasise that at every stage, as the Prime Minister has said, our planning and delivery is done in partnership with the states and territories. Each state and territory has different needs. It has different populations. It has different geography. And we need the expertise and the wisdom of the experts in each state and territory to do it. But the aged care and disability care will be a very high priority. The states and territories will set up dedicated clinics to do priority frontline health care workers, those people in the emergency departments and intensive care units who are at the frontline of exposure to COVID. And then particularly as we've seen more and more recently, those people working in our quarantine hotels, those frontline workers at the border and working in those quarantine hotels who are putting themselves probably at the highest risk of exposure of anyone in our community at the moment. And they will be a great priority. And again, we will do that in partnership with the states and territories.

So if you go to the next slide, the first stage will be, as the Prime Minister and Minister Hunt said, specific hubs for the minus 70 [degrees celsius] Pfizer vaccine, that will need to be a limited number of sites set up by the states and territories in hospitals, 30 to 50 across the nation. And they will remain Pfizer hubs the whole way through. So we don't have any confusion about which vaccine is available at which site. Each hub, each vaccination site will only deliver one type of vaccine. We've seen already some parts of the world where there might be some confusion about which dose of which vaccine you get. We want, because we're in this luxurious position of being pretty COVID free at the moment with a good COVID response, we want to vaccinate our population properly with the right dosage interval and give them two doses of the same vaccine according to the best possible protocols. Those hub locations will be determined by the states and territories in partnership with the Commonwealth, and they will deliver some vaccines to those frontline border workers, quarantine workers, frontline health care workers and become a distribution site for the outreach teams that will go into residential aged care and into disability care to deliver those vaccines to that initial priority population.

Then if we go to the next slide, you'll see a very large number of vaccination sites. This is within a few weeks of commencing vaccination. When we broaden to get access to the large quantity of the AstraZeneca vaccine, which we are, as I said, producing locally, initially we will be using imported doses and then we will have significant product coming off the production line at CSL's plant in Parkville, and we have enough plan to cover the entire population with two doses, and you'll see that we will expand our vaccination sites, in addition to those Pfizer hubs we will be expanding into general practice clinics, our existing Commonwealth GP respiratory clinics and a number of other state run vaccination clinics that will be determined in partnership with the states and territories and obviously also Aboriginal controlled health services. And you will see potentially a thousand or more sites across the country that will be rolling out vaccine for the subsequent phases. We will then obviously be expanding progressively those populations we saw on the first slide and over the course of this first- the second quarter of this year, we will have achieved a very significant proportion of the population coverage and then we will go on to cover the rest of the general population. And the very last group that we might consider is children. We know children are at the lowest risk of getting COVID and transmitting COVID. And the vaccines currently haven't been properly tested in children. And that will be the last group that we will consider in the fifth phase.

So you've seen all that you I think you will get access to these slides. And I'm happy to take questions later on. But I think Professor Kelly is going to put it into the general public health context now.

PRIME MINISTER: Thank you, Professor Kelly?

PROFESSOR PAUL KELLY, CHIEF MEDICAL OFFICER: Thank you, PM.

So all along through this pandemic, we've had a plan for dealing with the pandemic in an Australian way and the other speakers before me have spoken to that already. So vaccines fit into that plan. They are part of, it's a strong tool, it's not a magic bullet, it's not a silver bullet, as the PM has said, but it fits into and strengthens all of those things that we have in our pandemic control strategy. It will continue to include all of those personal behaviour issues that people know all about right about now. It will include the test, trace and isolate component and the excellent work being done in the states and territories right now dealing with the outbreaks that we have in Sydney and Melbourne and other places. So it's part of that story. It's a very exciting new part of that story. We said very early on it would take us probably 18 months to two

years to get to get a vaccine into Australia. We are well ahead of that initial phase. And it's an extraordinary position that we are in to be able to introduce that in the coming weeks. We are looking to save lives and also minimise the risk to the Australian population and others, Professor Murphy and Minister Hunt and the Prime Minister have talked about that importance of priority populations, and that fits into the plan that we've had all along. It's about those that have the highest risk of exposure for their own personal health, but also how that may transmit to the rest of the population. And so that's an important component. It's why quarantine workers and other border workers are right at the top of the queue right now. They are the ones at the highest risk, but it's also about protecting lives and protecting people at the highest risk, therefore, of severe disease. The elderly, particularly those in aged care facilities, some disabled people, Australian, Aboriginal and Torres Strait Islander people and those with chronic diseases. That's why they're in that priority group. It's because they are at the most severe end of the spectrum in terms of disease. And the important thing to note is both of those vaccines that are now on the cusp of being approved for use in Australia have shown very strong effectiveness against severe disease in particular. And so that's an important component of our plan.

Thank you, PM.

PRIME MINISTER: Let's keep questions at first to the vaccination strategy and those issues. Happy to deal with other issues later and matters for tomorrow. But as I said, there's not much more I can add about tomorrow's meeting until they've actually met.

JOURNALIST: So on the vaccine, can you take us through what's happened over the last 24 hours? Because the timetable has obviously bounced around a bit. You know, in November, you said that we'd be at the front of the queue. Then you changed it to saying we'd be on the front row. Yesterday, we had Greg Hunt saying the rollout would start in March. What's happened in the last 24 hours to bring it forward? It does sound a bit like you're taking the timetable that Labor has proposed. And when can that batch testing start? Can it only start, you know, after the TGA has approved it, then it's been introduced. Just explain to us why we had a two week concertining of that in 24 hours?

PRIME MINISTER: Well, let me start and then Greg and I think Brendan and others can comment. First of all, our practice has been to set out cautious targets. And that's what we've done with this. We talked about the first quarter of this year when we first talked about vaccines. And since that time, we've just worked the system to see how we can improve the timeliness of the delivery of the vaccine. So these are not things that have been decided in 24 hours. These are things that have taken us weeks to arrive at from the time that we agreed the national vaccination policy and then took that through National Cabinet. Health officials have been working with their counterparts and states, with our suppliers and others to continue to improve on the timetable that we have outlined. Now, we're not going to go out and make commitments against which we can't be confident of meeting. And that's why you've seen a gradual improvement of the position as we've set it out. The comparison that we have with other countries are with countries that are not in emergency vaccination territory. If you compare us to countries like Japan or South Korea or indeed New Zealand or Taiwan or other places, then that that's the group you've, I think everyone has agreed is a group that's good to be in when it comes to the impact of COVID-19 and the pandemic. And so we haven't gone to emergency vaccination arrangements for the reasons that Professor Murphy has outlined. And I think Australians want to, a swiftly developed and administered vaccine. But more importantly, they want a safe one, and they don't want any corners cut on that process. And that's certainly not what Australia would do. The Australian way is to follow the processes. Now, in following those processes to the letter and giving it the priority that we have has meant we've been able to continue to bring forward that date. But I do put this very strong qualification. The data has to match up and the data has to meet the requirements of the TGA. And to go to your point about the batch testing, that can only occur, as I understand it, once they have been delivered, and that is at least around about two weeks after TGA approval has been provided. So there are a number of steps that you must go through before I can, and others who were in that priority group can go and get that initial vaccination and then the follow up vaccinations that come.

But I'll ask Greg and I'll ask Brendan to also add to the answer?

JOURNALIST: You can get it, you can get the approval in late January, but you can roll it out in mid-February. Where does the batch testing?

PRIME MINISTER: Well, that's what I just said Samantha. There is a two week process for the vaccines to then be delivered to Australia because Pfizer, their global rule is that they deliver it around two weeks after TGA approval has been provided. The vaccines don't turn up before the TGA approval. They turn up two weeks after and then after that there is up to a week involved in batch testing of the doses that have been delivered. Greg?

THE HON. GREG HUNT MP, MINISTER FOR HEALTH: Look, our approach has been very clear. The principle of under promise and over deliver, which means that whilst we have allowed for cautious timeframes, if the because we're relying on data from others, shipping from others, testing from others or distribution from others, we've allowed for any gap in that chain. But as each of those elements has been ticked off, we've been able to move forward from the second half of the year to late in the first quarter. The fact that we've been able to get confirmation that the Pfizer data is going to be available soon, confirmation that now the TGA, this is actually something which has changed on my advice from Professor John Skerritt, that's the change in the last 48 hours, that the TGA will therefore be able to assess that at potentially an earlier timeframe and confirmation that the shipping arrangements are all on schedule mean that we can now provide a confident timeframe at an earlier time. But unless we were confident, we wouldn't want to take the risk. We've seen some jurisdictions, because of the emergency circumstance, not be in a position where they can necessarily meet all of the timeframes that had been set out. And our approach has been to look at being in the same category of timeframe as New Zealand, South Korea, Japan, Taiwan. And when you look at the arguably the most successful countries in the world in dealing with it, they've taken a very considered but expedited approach and that's what we've done on batch testing. But Brendan?

PROFESSOR BRENDAN MURPHY, SECRETARY OF THE DEPARTMENT OF HEALTH: I can add only very little. The key issue is that we've had increased certainty of stuff that's coming from the companies, particularly. Increased certainty on how we can expedite our process. We've been working on this for over a month. We've said where can we get better information, more robust data that will satisfy the TGA? How confident are we that that data will satisfy the processors? Can we give greater confidence to Government that we won't be setting dates that will not be realised? I should emphasise one other point that this time has not been wasted. We are doing 24 hour a day preparation with the states and territories on the most complex logistical exercise that perhaps we've done in public health in this country. We've seen in other countries where some of that logistical exercise, because they've had to roll out very quickly, has not been up to speed. We want to make sure that everything is set to go so that when we start vaccinating, we have a well-oiled machine that delivers exactly as we've promised. So we have been continually striving to improve our timelines and this is and this is an iterative and ongoing process.

JOURNALIST: On the UK variant, I know you can't foreshadow tomorrow, but have there been any discussions with any of the gentlemen here about restricting the border with the UK or separate quarantine arrangements?

PRIME MINISTER: They are all issues of the AHPPC are considering now and, of course, I discuss those matters with my own Chief Medical Officer. The day doesn't go past when we or the Health Minister and I don't speak and, you know, quite extensive briefings are held every couple of, three times a week and sometimes even more regularly than that with a broad field which includes Home Affairs, Foreign Affairs and a range of others. So these are matters we've been discussing. I'm looking forward to AHPPC finalising their recommendation later today and that means we can receive it hopefully this evening and consider that overnight and be in a position to make some decisions tomorrow. But my expectation is, given what I think has been a very good collaborative process amongst the Chief Health Officers, that if they are in agreement, then that should enable us, I think, to move through that issue fairly quickly tomorrow.

JOURNALIST: Prime Minister, how will the vaccine distribution to the states work? Will that just be on population or risk level will be taken into account and just separately on the use of the GPs to deliver the vaccines, we've seen in New South Wales reports that even though the COVID test is supposed to be free, people are being charged for GP consultation fees to get the test. What guarantees can you give around the price of the vaccine and its delivery?

PRIME MINISTER: I'll let the Secretary.

PROFESSOR BRENDAN MURPHY, SECRETARY OF THE DEPARTMENT OF HEALTH: So we can guarantee that the vaccine will be free and it will be delivered free. We're working with the medical bodies around how that will be achieved, but we do not want their cost to be any barrier whatsoever to access to the vaccine. The Government has committed to free vaccination and we will deliver that. In terms of delivery to the states, that will depend on those priority populations as a careful logistic planning exercise, looking at the populations in each state, and we will be delivering obviously just in time stock to meet those priority needs. So there's a very careful and complex planning process that's going on, looking at the populations of each of those at risk groups in each state and delivering the necessary vaccines according to that priority list.

JOURNALIST: Prime Minister, just on what Andrew was asking before, if I could just ask the chief medical officer, is it your opinion, Professor Kelly, that there is any need to close off the border to people coming from the UK or is testing required, extra testing on departure? And Prime Minister, if I can ask you, would you be open to the idea at all using Defence sites as quarantine bases to bring more Australians back?

PRIME MINISTER: Well, again, what we've discussed around the National Cabinet table is the most effective way to get people back is to use the hotel quarantine system that has been in place all year. That's where the greatest capacity is. That's where you can get the greatest application of both health support as well as the necessary security support, which is been delivered still significantly by the Australian Defence Forces. We have opened up additional sites such as in the Northern Territory and in Tasmania to supplement those arrangements. But that is what has enabled us to get more than 60,000 Australians home by the end of last year from late September, more than double what we'd set as a goal for that period of time. And so we are expanding further the ability for people to come back from overseas. I had a very positive discussion with the Victorian Premier about that earlier this week. They're gearing up to post-Australian Open. They've got that challenge to get through over the next month. And then, you know, their confidence continues to strengthen around those arrangements and we'll have further discussions on what that means for those arrangements tomorrow, that is the quarantine arrangements in the end to end processes. That should see us in February also for Victoria to be able to lift from where they currently are now. Getting Australians home is important, but keeping Australians safe and ensuring the robustness of our processes and the quarantine arrangements, particularly with the additional risks that come with the more contagious strain. That is obviously what has prompted my request to the Chief Medical Officer this week and the things that they're considering now. But I do think it's important to allow them to have those discussions and for the Chief Medical Officer to be able to have those discussions with them before I think he has them publicly. But Paul can speak for himself.

PROFESSOR PAUL KELLY, CHIEF MEDICAL OFFICER: Thank you, PM. So the AHPPC formally looked at the UK strain and made suggestions about what, if anything, should be done differently for the people coming from the UK before Christmas. Since then, we've met actually every single day, including Christmas Day and all of all through the Christmas New Year period. At every meeting we have discussed this issue and there is a specific paper that we worked through yesterday in relation to the UK strain, and that will be coming to the National Cabinet once it's gone through the AHPPC process. Just to say, though, that the UK strain is more transmissible. It is able to be, is more infectious to other people, therefore. It's become, it started in the south east of the UK. It is now pretty much the only game in town in the UK. It's been found in multiple other countries and it's been found here in hotel quarantine. So it is spreading. We have so far kept it under control within hotel quarantine and it is not transmitted when you're in your hotel room for 14 days. We know that hotel quarantine is safe. We need to, though, look at what else, if anything, needs to be done and that is what will be discussed at National Cabinet tomorrow.

JOURNALIST: You mentioned a discussion with the states about when and how vaccines could be required by uniform public health orders. Could you please clarify who that would apply to? Would that be aged care workers, returning Australians? And how does that square with repeated assurances that vaccines will be voluntary?

PRIME MINISTER: All I've said today is that that is a discussion that needs to be had. And so we will have that discussion.

JOURNALIST: But you said it would be voluntary, so...

PRIME MINISTER: It is voluntary. But that is an important discussion for the public health and safety that needs to be had in the states and territories who are responsible for public health.

JOURNALIST: So you're leaving open the possibility that you could require certain cohorts of people like returning Australians, aged care workers...

PRIME MINISTER: We're going through the processes that are necessary to protect public health and safety in the country and I'm doing that collaboratively with the states and territories, who are principally responsible for that, John.

JOURNALIST: Is that going back on your previous statement...

PRIME MINISTER: No, I think you're over interpreting, Paul. You've got a want to do that. Yep.

JOURNALIST: Prime Minister, does Donald Trump bear some responsibility for undermining democracy and some of the chaotic...

PRIME MINISTER: Let's just stay with the vaccination program. I'm happy to come to other issues. Yep?

JOURNALIST:, Prime Minister one, would you use ADF capabilities to ensure that this vaccine is swiftly delivered in time for winter? But two, I hate to be ungentlemanly, but I couldn't notice that your age bracket is in phase 2.A. Would you and members of the Cabinet be vaccinated earlier? And would you be willing to do that on television in the same way that Joe Biden and Kamala Harris were?

PRIME MINISTER: Well, in answer to your last question, yes. And I think it's important for public confidence for leaders around the country to do that and I'll obviously make myself available to do that. And, you know it's a discussion we'll have with Premiers, but I have no doubt that they would equally be stepping forward. We've had those discussions around the National Cabinet table before. It's not a new idea. It's not the most important issue obviously, what's important is the other things that I think have been discussed today. But there's no reticence there. We need to be mindful of the, you know, the doses that are available. As I said, were starting at around about 80,000 a week and that will scale up, as it is in all countries. It starts out in all countries at a limited amount and then as, as are the more doses become available, and particularly when the AstraZeneca vaccine becomes available, then it ramps up from there. But I think for the sake of public confidence and I have absolutely no issue with that, I have total faith in the Therapeutic Goods Administration. I trust them with the decisions about the vaccinations that I give to my children on all other matters and so I think Australians rightly have a lot of confidence. But Brendan can deal with the other matters.

PROFESSOR BRENDAN MURPHY, SECRETARY OF THE DEPARTMENT OF HEALTH: So we have contracted some very experienced logistics providers to do logistics. We're confident that the providers we've got at the moment, working in close partnership with the states and territories who are well experienced in vaccine distribution, will meet our needs. The ADF has been through us right through this pandemic, if we've ever needed them there to step up and help and they are incredibly versatile and would be able to do so if required, but we don't think it will be required. We think that our logistics providers are up to the task.

JOURNALIST: Sorry, Prime Minister, just to clarify, would you and members of the Cabinet get vaccinated in earlier phases, in phase 2.A, considering most of you are in the phase 2.A age bracket.

PRIME MINISTER: The Health Minister and I will certainly line up for that. But I mean, I think that's necessary to show public confidence and so we're comfortable with that. I don't think we need to line the whole Cabinet up, by the way. I think, you know, there are more important people who need to get vaccinated, frankly, than me and the Health Minister, and the Premiers, for that matter. They're the aged care workers, they're the frontline health workers, they're the people working in hotel quarantine on our borders, they're the elderly who are in the disability sector and the vulnerable in those areas that we've identified. That's the priority. We're not the priority. But I think it is important for a show of public confidence and that's been done in other places and we'll certainly line up for the televised jab. I'm sure you'll all enjoy it.

THE HON. GREG HUNT MP, MINISTER FOR HEALTH: And, look just very briefly on that. We had given that some thought. None of us wanted to be queue jumpers, but we also recognise the public confidence role. And so we've come down on the side that the Prime Minister, the Health Minister, maybe a few others, but not the Cabinet as a whole. We don't believe that we qualify on the public health grounds. It's very important we recognise that but it does matter that we have public confidence and that's why we would look to do it with both the Government and Opposition Leaders and I've had that discussion with Chris Bowen, very constructive looking at doing it at a comparable time. So it's about the confidence and trust of the leaders and bipartisanship, but not about the Cabinet as a class.

PRIME MINISTER: Let's let everyone have a go, then I'm happy to...

JOURNALIST: Prime Minister, with regards, we've mentioned that the priority population will start being vaccinated between February and March. When can the general population expect to be off to receive the vaccine and how will that work?

PRIME MINISTER: Well, I'll ask Brendan to add to this, but what we have learnt all the way through is you've got to act on the best information available to you. You've got to do with the priorities straight away and that's what we're talking to you about today. We've identified three other priorities, three other areas of the population that it will move to once we've been able to work through those first two priority populations. Remembering in those populations, you've got to do two doses over about around about a month or so, in some cases less than that period of time. And so they provide the first ring of containment and protection of the vulnerable and that's where our priority has to be. And we will be able to update again, just as we have today, on further timetables for further populations once we're in a stronger position to be able to do that and that will depend on further information that comes forward. It will depend on the approvals, meeting the timetables that we've talked about and the production schedules, and the roll out of the distribution mechanisms which are well underway. I mean, one of the things that have been happening over this period of time is things like workforce readiness, an important part of the distribution process already underway. That's not something that has to wait for TGA approval. That's something that is already being done as well as the vaccination kits and things of that nature, ensuring we have enough syringes and all of that. That can be done now, that is being done now. But there are certain things that have to only take place and can only take place once the TGA approval is in place and that is a matter of weeks, both in terms of the delivery of the vaccines and the batch testing that needs to take place and the transport logistics, obviously, of them getting them to those many hospitals, public hospital locations around the country. Brendan?

PROFESSOR BRENDAN MURPHY, SECRETARY OF THE DEPARTMENT OF HEALTH: So you'll see from the slide that nearly half the population are actually covered in those initial priority populations and that includes a number of people who would not be considered at risk, but they might be frontline health care workers. They're not medically at risk. So we'll be covering a significant proportion of the population in those first priority phases and hopefully that will be underway in that, you know, that March quarter. And as the Prime Minister said, it depends how quickly that goes how soon we can then broaden that out towards the middle of the year to the broader general adult population.

JOURNALIST: Where will most Australians get their vaccines? Will you be commandeering town halls, schools, that kind of thing?

PROFESSOR BRENDAN MURPHY, SECRETARY OF THE DEPARTMENT OF HEALTH: So there will be a number of sites. The, obviously those Pfizer hubs will be initially focused on that high priority population. So that'll be a small proportion. The majority of the population will get their vaccines from either what we call a GP respiratory clinic, which the Commonwealth has set up. Some general practises who want to participate and can deliver the vaccine program and they will be selected on that basis. The states will be setting up some special vaccination clinics in addition to the Pfizer hubs. Aboriginal controlled health services will be setting up. Towards the middle of the year when we're broadening beyond the priority population, Minister Hunt has already committed that we will be looking at arrangements that pharmacies may be able to also be rolled out in the second half of the year. So most of the population will probably get it at one of those settings.

JOURNALIST: At the moment, there's issues getting fresh food and vegetables etcetera to remote indigenous communities. How are you going to roll out the vaccine in cold temperatures to remote indigenous communities?

PROFESSOR BRENDAN MURPHY, SECRETARY OF THE DEPARTMENT OF HEALTH: Well, it's obviously a huge logistical challenge if indeed we are choosing to use a minus 70 vaccine. We do have a vaccine, the AstraZeneca vaccine, which is very stable in normal cold chain refrigerator temperatures and is likely to be the priority vaccine for those settings. So that's been worked through. We've got a very specific plan, working with the Aboriginal control sector on how we're going to vaccinate remote indigenous communities. But it's likely that we won't be prioritising the use of a difficult logistic vaccine for that population.

THE HON. GREG HUNT MP, MINISTER FOR HEALTH: I might just add something here. I think it's a very important question. One of the extraordinary achievements of Australia's vaccination program is that whilst the five year old vaccination rates through the course of COVID in 2020 increased from, to 94.8 and then 94.9 percent, Indigenous Australian vaccinations for five year olds are at almost 97 per cent and so they are higher than the national average of vaccination. So Indigenous Australia has been extraordinary in taking it up and that includes within remote communities. And so that remote community network through the Aboriginal Community Controlled Health Services is a fundamental part of the distribution network.

JOURNALIST: This has been a big debate in the UK about the distance between people being vaccinated. Obviously, we're not in that situation. You know, they're rolling it out really quickly because of the situation in the UK. But here in Australia, how long will it be between the first jab and the second jab? And what do you think about the British proposal to wait up to three months for the second jab?

PROFESSOR BRENDAN MURPHY, SECRETARY OF THE DEPARTMENT OF HEALTH: Look, I think we're in a very, very different situation. In the UK they have a significant public health crisis. And they've decided that because the first dose provided some protection, that they will compromise what would be an ideal vaccination strategy. We're not in that position. We fully understand and support what they've had to do. We are planning to go with the trial evidence around two doses of the same vaccine at roughly a month apart. Obviously, you can have a bit of slippage either way, but we want to make sure that people come back in that, in that recognised time frame to get their second dose. We want to have the systems to make sure that they can come back to the same clinic, have the systems to call them back, have the systems to record their data so that we know exactly what they've got, when they've had it, and to fully vaccinate people in an appropriate and staged timeline. That's why so much planning is happening at the moment. We have had, we've got a team of 60 people in the health department and another team of similar size around the government who are working, and a lot of consultants and contractors working on this logistic planning so that we, it works smoothly from the minute we start.

JOURNALIST: On the Pfizer hubs, can you just explain are those 30 to 50 hubs, are they just warehouses or are they going to be locations where people will go to?

PROFESSOR BRENDAN MURPHY, SECRETARY OF THE DEPARTMENT OF HEALTH: They'll be locations where some people will go for example, the quarantine and border force workers and the frontline health care workers will go to those sites to get their vaccine. But they will also distribute or provide vaccines for outreach teams to go into aged care facilities and disability residences, to vaccinate residents in those places. So they will do both.

PRIME MINISTER: For me, I will pop down to the Canberra Public Hospital. Any other questions on the vaccination strategy or those issues relating? Very good, well on that basis I might excuse the Chief Medical Officer because he's got a very important AHPPC meeting to rejoin. And thank also Professor Murphy for the great work he's been doing and leading a team of experts that has been in place for some time now since the, we formally approved the national vaccination policy that was pulled together by Professor Murphy. And that has been critical to running this process to where we are now. John?

JOURNALIST: Thanks PM. Does Donald Trump bear some responsibility for undermining democracy and inciting some of the chaotic scenes we've seen in the US Capitol building overnight?

PRIME MINISTER: Look, I'm not going to offer any more comment than the one I've already made on this issue. I think what we've seen in the United States is terribly distressing, terribly concerning. And I noted the President's message this morning to tell people to go home peacefully. I hope that's what people are doing. And this is a very distressing time in the United States. And I feel terribly for what's happening there. I really do. I mean, they are one of our closest friends and partners, the United States. And I feel for those here in Australia who have family in the United States or maybe that's their origin and it's heartbreaking to see what's happening there. This is one of the world's greatest democracies, and it remains so because of its forbearance and its persistence in the principles and values upon which the nation was established. And I have no doubt that that will prevail.

JOURNALIST: In that same video message, though,

PRIME MINISTER: I'm not here to offer a running commentary on what should be happening in the United States. What I'm here to do is ensure that we deliver our vaccination plan. What I'm here to do is ensure that this year the Australian economy continues to recover, that we get people back into jobs, and that we can ensure that the Australian economy and society again, can open up as safely as we come out of this holiday period, that people as they come back from their leave, can get back to work and we can have a 2021 that is certainly much better than 2020. But we're not out of the woods yet. We haven't won yet, but we're certainly winning a lot better than a lot of other places around the world. And we want to make sure we maintain that and the way I do that is I stay focussed on what's happening here in Australia and the needs of Australians. And that has my absolute priority focus.

One at a time.

JOURNALIST: In that same video message, the President of the United States of America, while telling people who had trashed the Capitol building to go home, said, we love you, you are special people. Is that a responsible message from a world leader?

PRIME MINISTER: It's not for me to offer commentary on other leaders. I don't do that out of respect for those nations. And that's where I'm going to leave that matter. I've expressed my great concern and distress about what has been happening in the United States, just as other leaders of the world's democracies have and I concur with their view.

JOURNALIST: Is there any change to our diplomatic arrangements in Washington, DC given our proximity, given the proximity of the US- the Australian embassy, to some of those violent protests?

PRIME MINISTER: No, not at this point, but we'll obviously take advice on those things. And I'm sure that the Secretary of DFAT will continue to advise on, on anything that is necessary, as they would in relation to any Post. But there is no advice along those lines at this point.

JOURNALIST: Prime Minister, after the US election, George Christensen promoted false claims of dodgy extra votes for Joe Biden. I accept you don't want to comment on Trump, but will you condemn conspiracy theories being promoted by members of your own Government?

PRIME MINISTER: You know, Australia is a free country. There's such a thing as freedom of speech in this country and that will continue.

Okay, well thank you all very much. I'll see you here tomorrow after the AHPPC, thank you, has met and the National Cabinet has met in the morning. Thank you.

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